MEDICAL RECORD	**CONSULTATION SHEET**

REQUEST

TO: PULMONARY	FROM: *(Requesting physician or activity)* HOURI, GOLD	DATE OF REQUEST 11/3/97

REASON FOR REQUEST *(Complaints and findings)*

64 y/o Recent CT chest smi RLL subpleural irregular nodule 2° to scarring or malignancy please advise as to further evaluation. (He has had prior CTs from an outside hospital which I will attempt to obtain)

PROVISIONAL DIAGNOSIS

RLL nodule

DOCTOR'S SIGNATURE	APPROVED	PLACE OF CONSULTATION ☐ BEDSIDE ☐ ON CALL	☐ ROUTINE ☐ TODAY ☐ 72 HOURS ☐ EMERGENCY

CONSULTATION REPORT

(Continued on reverse side)

SIGNATURE AND TITLE	DATE

IDENTIFICATION NO	ORGANIZATION	REGISTER NO	WARD NO

PATIENT'S IDENTIFICATION *(For typed or written entries give: Name—last, first, middle; grade; rank; rate; hospital or medical facility)*

DANKOJR, PAUL
169-26-7266

CONSULTATION SHEET
STANDARD FORM 513 (Rev. 9-77)
Prescribed by GSA/ICMR
FIRMR (41 CFR) 201-45.505
513-108

PULMONARY [illegible]

EMERGENCY TESTI[illegible] is performed only by prior appr[illegible]l

MEDICAL RECORD | **CONSULTATION SHEET** 12/26

REQUEST

TO: PULMONARY LABORATORY

FROM: Pulmonary

DATE OF REQUEST

REASON FOR REQUEST (Complaints and findings)

PULMONARY FUNCTION STUDY REQUEST

PATIENT INFORMATION

1. Chest X-ray: NEG ( ) INFILTRATE ( )
2. Using supplemental oxygen ( ) Yes ( ) No
3. If Pre-op, indicate date ________
4. PULMONARY DIAGNOSIS:
5. History of occupational exposure:

Hb: ________ (If diffusing Capacity desired)

TESTS REQUESTED

(✓) SPIROMETRY (FV LOOP, FVC, FEV_1, FEV_1/FVC RATIO)
( ) LUNG VOLUMES (RESTRICTION SUSPECTED
(✓) DIFFUSING CAPACITY
( ) BRONCHODILATOR
( ) ARTERIAL BLOOD GASES
( ) HOME O_2 EVALUATION
( ) O_2 STUDY PATIENT
( ) MVV (For study patients only)

DOCTOR'S SIGNATURE (PRINT NAME)	APPROVED	PLACE OF CONSULTATION ○ BEDSIDE ○ ON CALL	○ ROUTINE ○ TODAY ○ 72 HOURS ○ EMERGENCY

CONSULTATION REPORT

PATIENT EVALUATION (PERFORMED BY PFT STAFF)

Smoking History: Currently smoking (✓) Cigarette <192 pack-years. Cigar/Pipe ______
Non-Smoker ( )
Quit ____ years ago, ______ months ago, ______ weeks ago

Cough: History of cough: Months/[✓ Years] < 20 None ______ Sputum color [illegible]
Test: Loose (✓) Dry ( ) None ( )

Dyspnea:
( ) 0 - None
( ) 1 - One flight or walking fast
(✓) 2 - Walking on level ground
( ) 3 - Sitting in chair
( ) 4 - Sometimes

Breathing Medications: N

Time of last dose ______ Hr/ago

Maximum Pressures: Inspiration ______ Expiration ______

Patient effort and cooperation:

(Continued on reverse side)

SIGNATURE AND TITLE	DATE

IDENTIFICATION NO	ORGANIZATION	REGISTER NO	WARD NO

PATIENT'S IDENTIFICATION (For typed or written entries give: Name—last, first, middle; grade; rank; rate; hospital or medical facility)

OP-264 (646)

DANKO, T266

169-26

CONSULTATION SHEET
STANDARD FORM 513 Rev.
Prescribed by GSA/ICMR
FPMR 101-11.806-8

Source: https://www.industrydocuments.ucsf.edu/docs/gqgg0072

ULMONARY FUNCTION LABORAT
V.A. PITTSBURGH HEALTHCARE SYSTEM
UNIVERSITY DRIVE DIVISION

Name:DANKO, PAUL JR.		ID#:169-26-7266
Age:	64 Years	Room:PLM Date:09-DEC-97
Sex/Race:	Male / Caucasian	Temp/Pres: 23 C / 733 mmHg
Height:	67 in 170 cm	Physician:ATWOOD
Weight:	176 lbs 80 kg	Tested by:LARRY ALEXANDER

			PRE-RX		POST-RX		
		PRED	BEST	%PRED	BEST	%PRED	%CHG
SPIROMETRY (BTPS)							
FVC Liters		4.07	2.51 #	62*			
FEV1 Liters		2.86	1.93	68*			
FEV1/FVC %		71	77				
FEF25-75% L/Sec		2.78	1.83	66			
FEF50% L/Sec		3.76	2.62	70			
PEF L/Sec		7.76	7.00	90			
Vol Extrap Liters			0.13				

------

* = OUTSIDE 95% CONFIDENCE INTERVAL # = OUTSIDE NORMAL RANGE
CALIBRATION: PRED: 7.04 ACTUAL: EXP [illegible] INSP [illegible]
IPS-OL10-08 IPS-OH10-07 N-[illegible]

COMMENTS:

PT. EXECUTION THROUGHOUT TESTING VERY POOR. UNABLE TO OBTAIN RELIABLE DLCO

Source: https://www.industrydocuments.ucsf.edu/docs/gqgg0072

PULMONARY FUNCTION LABORATORY
V.A. PITTSBURGH HEALTHCARE SYSTEM
UNIVERSITY DRIVE DIVISION

Name: DANKO, PAUL JR.
ID#: 169-26-7266
Age: 64 Years
Room: PLM
Date: 09-DEC-[illegible]
Sex/Race: Male / Caucasian
Temp/Pres: 23 C / 733 mmHg
Height: 67 in 170 cm
Physician: ATWOOD
Weight: 176 lbs 80 kg
Tested by: LARRY ALEXANDER

SPIROMETRY (BTPS)		PRED PRE-RX	TRL1	TRL2	TRL3	BEST	%PRED	%CHG
FVC	Liters	4.07	2.21	2.51	2.42	2.51*	[illegible]	
FEV1	Liters	2.86	1.45	1.77	1.85	1.93	[illegible]	
FEV1/FVC	%	71	66	77	76	77		
FEF25-75%	L/Sec	2.78	0.66	1.83	1.50	1.8[illegible]	[illegible]	
FEF50%	L/Sec	3.76	1.59	2.62	2.99	2.62	[illegible]	
PEF	L/Sec	7.76	1.99	3.19	7.00	7.0[illegible]	[illegible]	
Vol Extrap	Liters		0.06	0.13	0.05	0.1[illegible]		

------

* = OUTSIDE 95% CONFIDENCE INTERVAL   L% = OUTSIDE NORMAL RANGE
CALIBRATION: PRED= 3.00 ACTUAL= [illegible]
IPC-OL10-08 IPS-OH10-07 [illegible]

COMMENTS:

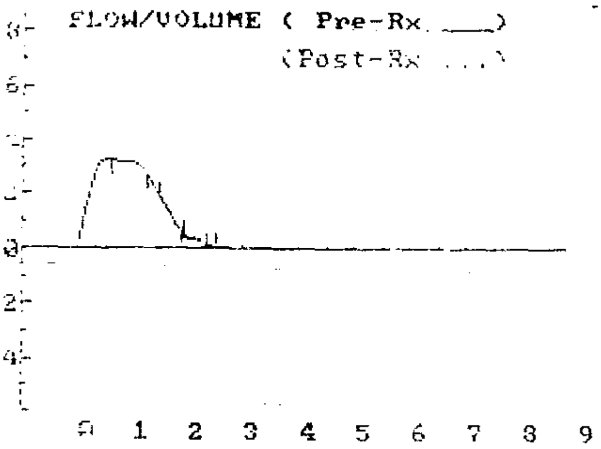
[illegible] EXECUTION THROUGHOUT [illegible] VERY POOR, [illegible] RELIABLE DLCO

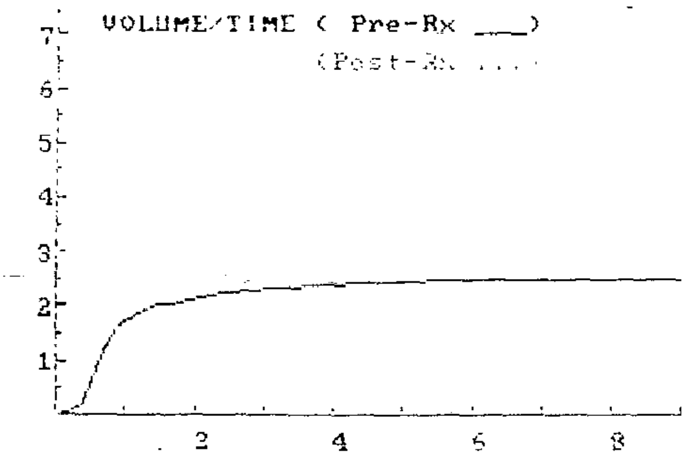
PULMONARY FUNCTION LABORATORY
V.A. PITTSBURGH HEALTHCARE SYSTEM
UNIVERSITY DRIVE DIVISION

Name:DANKO, PAUL JR.		ID#:169-2[illegible]-7246
Age:	64 Years	Room:PLM Date:09-DEC-97
Sex/Race:	Male / Caucasian	Temp/Press:23 C / 7[illegible] mmHg
Height:	67 in 170 cm	Physician:ATWOOD
Weight:	176 lbs 80 kg	Tested by:LARRY ALEXANDER

SPIROMETRY (BTPS)			PRE-RX		POST-RX		
		PRED	BEST	%PRED	BEST	%PRED	%CHG
FVC	Liters	4.07	2.51 #	62*			
FEV1	Liters	2.86	1.93	68*			
FEV1/FVC	%	71	77				
FEF25-75%	L/Sec	2.78	1.83	66			
FEF50%	L/Sec	3.76	2.63	70			
PEF	L/Sec	7.76	7.00	90			
Vol Extrap	Liters		0.13				

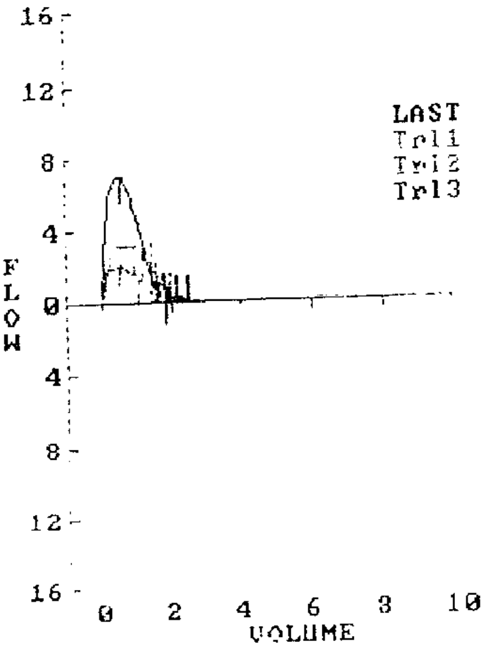
= OUTSIDE 95% CONFIDENCE INTERVAL * = OUTSIDE NORMAL RANGE
[illegible] IPS-0810-07 [illegible]





Source: https://www.industrydocuments.ucsf.edu/docs/gqgg0072

Name: DANKO, PAUL JR.
ID #: 139-26-7266



1P

MEDICAL RECORD | PROGRESS NOTES

DATE 10/27/97 Ent

F.V. Ⓡ Neck Mass – Resolved

– Prob LN – FNAB ⊖

Mass is gone –

was "2+3

Still Swol ~~to 8 [illegible]~~

On Tagamet – still some [illegible]

Had Chest CT

"Lesions" done 7/1 ②

Last CT R

Ba Sw – ① Achalasia c big opening LES

② Esophgl dysfn

③ H. Hernia

④ small pharyngocele

Neg CT of Neck

FNAB ⊖

IDL – wl

OC – wl

Nock – Ill defined

Ⓡ Neck Fullness – smaller than when [illegible] palpated – posterior

Imp – Plan

No ED

GERD

GERD

Discuss smoking cessation

Plan Prilosec / Propulsid

(Continue on reverse side)

PATIENT'S IDENTIFICATION (For typed or written entries give: Name—last, first, middle; grade; rank; rate; hospital or medical facility)

Wanko, Paul

169-26-7266

REGISTER NO

WARD NO

PROGRESS NOTES

STANDARD FORM 509 (Rev. 11-77)

Prescribed by GSA/ICMR

FIRMR(41CFR)201 45 505

509 111

MEDICAL RECORD | **CONSULTATION SHEET**

REQUEST

TO: ENT

FROM: (Requesting physician or activity) ECC

DATE OF REQUEST 3/5/97

REASON FOR REQUEST (Complaints and findings)

63 yo c̄ long standing h/o [illegible] [illegible] [illegible]
choking, hoarseness – non smoker
h/o GERD, IV [illegible] ...

PROVISIONAL DIAGNOSIS

SCHEDULE AFTER 5/1/97

DOCTOR'S SIGNATURE Cox

APPROVED

PLACE OF CONSULTATION ☐ BEDSIDE ☐ ON CALL

☐ ROUTINE ☐ TODAY ☐ 72 HOURS ☐ EMERGENCY

CONSULTATION REPORT

Please See Progress Note
@ 7/7/97

(Continued on reverse side)

SIGNATURE AND TITLE

DATE

IDENTIFICATION NO.

ORGANIZATION

REGISTER NO. 169 20 7200

WARD NO.

PATIENT'S IDENTIFICATION (For typed or written entries give: Name—last, first, middle; grade; rank; rate; hospital or medical facility)

DANKO, JX [illegible]

CONSULTATION SHEET
STANDARD FORM 513 (Rev. 9-77)
Prescribed by GSA/ICMR
FIRMR (41 CFR) 201-45.505

Source: https://www.industrydocuments.ucsf.edu/docs/gqgg0072

MEDICAL RECORD | PROGRESS NOTES

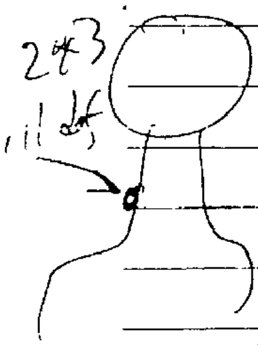
DATE

7/7/97 Ent - New PT / Consult
– Patient seen late (consult 3/97)
at his request –
64 y/o ♂ ~~[illegible]~~ w/ hoarseness /
dysphagia, occas. otalgia, mostly right
sore throat - left. Hx of [illegible] [illegible]

Tob - 4 packs/day
– Stopped 10 years
– ETOH ⊕

AD - WNL
AS - WNL
Nose - WNL - septal deviation to right.
cannot see Nasopharynx
Rt. post neck - ⊕ Node ? [illegible] [illegible]
F. no needle.

[illegible] [illegible]
– All: [illegible] [illegible]
Fx hx - [illegible] [illegible]
Bypass - 92.



No CXR in [illegible]

A/P. Hoarseness / dysphagia

FNA Rt. post neck Adenopathy
Videostrobe - Nl
Nasopharynx - Nl
1) Barium Swallow
2) CXR ✓
③ CT Neck
BUN [illegible]
④ [illegible]

STAT CXR Tod — Neg

(Continue on reverse side)

PATIENT'S IDENTIFICATION (For typed or written entries give: Name—last, first, middle; grade, rank, rate, hospital or medical facility)

Danko Jr, Paul
169-26-7266

REGISTER NO. | WARD NO.

PROGRESS NOTES
STANDARD FORM 509 (Rev. 11-77)
Prescribed by GSA/ICMR
FIRMR (41 CFR) 201-45.505
509-111

DEPARTMENT OF LABORATORY MEDICINE
SURGICAL PATHOLOGY REPORT

Specimen No. S92-1759

Name DANKO, PAUL   Age 58Y   Sex M   MR No. 428804

Pt. Loc. 37301   Ord. Loc. OR12   Received 02/18/92   Reported 02/19/92

Att. Phy. FRAZIER, JOHN E., M.D.   Ord. Phy. MAGOVERN, GEORGE JR., M.D.

Specimen: Right coronary artery plaque

Clinical Diagnosis:

MAC DES:

The specimen, right coronary artery plaque, consists of a 3.8 cm, elongated, calcified piece of tan-yellow tissue, sectioned. The specimen is totally submitted in one cassette.

DG/vap

DIAGNOSIS:

RIGHT CORONARY ENDARTERECTOMY:
CALCIFIED ATHEROSCLEROTIC PLAQUE

Robert R Dawson, MD
Robert R. Dawson, M.D.
Resident

Peter R. Olson, M.D.
Surgical Pathologist

cb

~~DANKO Exhibit No. 13-A~~   ~~[illegible] 12A-13~~

3242
HAHN,RICHARD M
FEO#: 1-084-127
ABDOMEN 2.01

REG:03/24/92
DOD:072033

FINAL RADIOLOGY REPORT

REPORTS TO:HAHN/BALK &.....
XRAY FILE#:37968
EXAM DATE:03/24/92

RADIOLOGIST: SINGH,HERAMB
STENOGRAPHER: COLLAR,JEANNETTE

CT SCAN OF THE ABDOMEN WITH CONTRAST AND LIVER WITH AND WITHOUT CONTRAST:

CT OF THE ABDOMEN AND LIVER WAS OBTAINED OF THE DOME OF THE DIAPHRAGM TO THE PUBIC SYMPHYSIS. THE STUDY DEMONSTRATES IN THE LIVER THERE IS NO EVIDENCE OF ANY DILATED INTRA OR EXTRAHEPATIC DUCTS THE GALL BLADDER APPEARS TO BE NORMAL. THE PANCREAS IS ALSO OF NORMAL DENSITY. THE SPLEEN IS NORMAL. BOTH KIDNEYS ARE NORMAL IN NEPHROGRAM. THERE IS NO EVIDENCE OF ANY CYST OR MASS. THE BOWELS ARE WELL OPACIFIED WITH ORAL CONTRAST AND THERE IS NO EVIDENCE OF ANY MASS SEEN.

IMPRESSION

NORMAL CT SCAN OF THE ABDOMEN AND LIVER WITH AND WITHOUT CONTRAST.

LAST PAGE

1992

[illegible] 05185340324[illegible] FINAL RADIOLOGY RE[illegible]

Source: https://www.industrydocuments.ucsf.edu/docs/gqgg0072

DIAGNOSIS (CONT'D):

3. Coronary artery disease:
    a. Significant disease of the proximal left anterior descending with a critical stenosis of the mid left anterior descending.
    b. Significant disease of the proximal left circumflex with a critical stenosis of the mid left circumflex.
    c. Functional occlusion of the distal right coronary artery.

COMPLICATIONS: None.

PLAN: Coronary artery bypass graft surgery, hopefully with grafts to the left anterior descending, mid and posterior marginals, and posterior deccending artery.

ATTENDING: Michael J. Pecora, M.D.

FELLOW:

Michael J Pecora MD

Michael J. Pecora, M.D.

MJP/mkm

DAr Exb No. 13

Source: https://www.industrydocuments.ucsf.edu/docs/gqgg0072

ALLEGH Y GENERAL HOSPITAL CARDIAC ABORATORY
CARDIAC CATHETERIZATION REPORT

PATIENT: DANKO, PAUL
DATE OF STUDY: 2/14/92

ID# 48 88 80
DOB: 7/27/33

FILM NO: JH 374

TYPE OF STUDY: SELECTIVE CORONARY ARTERIOGRAPHY AND LEFT VENTRICULOGRAPHY

PERFORMED BY: Michael J. Pecora, M.D.

MAR 1 7 199

ANESTHESIA: Valium, Xylocaine.

CATHETERS: #8 French multipurpose.

CONTRAST: Omnipaque.

INDICATIONS: Unstable angina, overwhelmingly positive exercise echocardiogram.

RESULTS: Left ventricle: Ventriculography demonstrates essentially normal overall left ventricular function, perhaps with mild inferior hypokinesis. The left ventriculogram was performed a hand injection, due to the patient's markedly positive exercise treadmill and critical triple vessel disease. There is no mitral regurgitation. The left ventricular end-diastolic pressure pre ventriculography was 24. The left ventricular end-diastolic pressure post nitroglycerin dropped to 14.

Right coronary artery: There is a 40% diameter stenosis of the proximal right coronary artery, a 40% stenosis of the mid right coronary artery, and a 99% diameter stenosis of the distal right coronary artery prior to the posterior descending artery. The distal right coronary lesion is a very long, complex stenosis. The posterior descending artery is a large bypassable vessel.

Left main: Normal.

Left anterior descending: There is a 60% diameter stenosis of the proximal left anterior descending with a 95% diameter stenosis of the mid left anterior descending. The distal left anterior descending is a large bypassable vessel.

Left circumflex: There is a 70% diameter stenosis of the proximal left circumflex with a 90% stenosis of the mid left circumflex prior to a large posterior marginal. The mid and posterior marginals are large bypassable vessels.

DIAGNOSIS:
1. Elevated left ventricular end-diastolic pressure.
2. Essentially normal overall left ventricular systolic function.

Danko Exhibit No. 13 D

Source: https://www.industrydocuments.ucsf.edu/docs/gqgg0072

ALLEGHENY GENERAL HOSPITAL
RECORD OF OPERATIC

PATIENT: Danko, Paul **MEDICAL RECORD #:** 488804

DATE: 02-18-92

SURGEON: George J. Magovern, Jr., M.D.
ASSISTANT: Robert R. Lazzara, M.D.
John Belhumeur, PA-C
ANESTHESIA: General endotracheal.
ANESTHESIOLOGIST:

PREOPERATIVE DIAGNOSIS: Coronary artery disease.

POSTOPERATIVE DIAGNOSIS: Same.

OPERATIVE PROCEDURE: Coronary bypass grafting LIMA to AD, SVG to circumflex marginal, SVG to RCA with endarterectomy of RCA.

CROSS CLAMP TIME: 17 minutes.
TOTAL BYPASS TIME: 111 minutes.
COMPLICATIONS: None.

OPERATIVE NOTE: The patient was brought to the Operating Room and identified, following which a Swan-Gantz catheter and femoral arterial lines were placed. The patient underwent general endotracheal anesthesia and was prepped and draped in the usual sterile fashion, following which a standard midline sternotomy incision was made and taken down through the subcutaneous tissues. The sternum was marked in the midline and transected. At the same time, a saphenous vein was harvested from the lower extremity in the usual fashion. After opening the sternum, bleeding was controlled, and the IMA tract was inserted . The IMA was taken down from the left internal thoracic bed. Upon taking down the IMA, it was felt to have a good pulse and be of adequate size. Attention was then turned back to the anterior mediastinum, where the pericardium split from the anomalent vein and the diaphragm marsupialized in the usual manner. To concentric pursestrings were placed in the ascending aorta. A single concentric pursestring was placed to the right atrial appendage. The patient was systemically heparinized. The arterial cannula followed by the venous cannula were placed. At this time, the IMA was prepared for distal anastomosis. The patient was the placed on cardiopulmonary bypass. The aorta was crossed clamped and cold blood cardioplegia was infusion antegrade through a 14 gauge angiographic catheter, approximately 1,000cc. was given. At this time, attention was turned to the right coronary artery. The proximal portion was dissected free from surrounding epicardial tissue. The vessel was heavily calcified. At this time it was decided to perform an endarterectomy. The vessel was opened for approximately 4cm and endarterectomized without any difficulty. An end to side anastomosis was the performed with the previously harvested, reversed, and beveled saphenous vein graft utilizing 7-0 Prolene. The vein was marked for length and transected. Approximately100cc. of cold blood cardioplegia was given down this vein. Two repair stitches were made, one on each side. A this time, attention was turned to the obtuse marginal branch #1. The circumflex coronary artery was dissected free from surrounding epicardial fat. An arteriotomy was made, and the vessel was free of disease. End to side anastomosis was then performed with running 7-0 Prolene without difficulty. The vein was marked for length and transected. At this time, attention was turned toward the left anterior descending portion. The left anterior descending artery in its mid portion was dissected free from surrounding epicardial fat, just distal to the crossing vein.

~~DANKO EXB No. 13 — E~~

The vessel was [illegible] it easily too[illegible] 1½mm probe, distally, as we[illegible] as proximally. An end to sid anastomosis was then performed to the left internal mammary artery utilizin running 8-0 Prolene. A single repair stitch was performed on the left latera side. At this time the aortic cross clamp was removed. The partial occlusio clamp was placed. Two 4mm punches were made and proximal anastomoses performe with running 6-0 Prolene. The partial occlusion clamp was removed, and the vein were aspirated of air. Please note that prior to cross clamping the aorta, pulmonary vent was placed to the main pulmonary artery. At this time it wa removed, and the hole oversewn with running 4-0 Prolene. Atrioventricular pacin wires were placd in the standard fashion. A left pleural chest tube was placed The patient was then slowly weaned from cardiopulmonary bypass without difficulty Following a good cardiac index and mean pressure range of 70, venous cannula wa removed and the arterial cannula was used for transfusion of pumped blood Protamine was begun. Following institution of Protamine, the arterial cannula wa removed. The site was oversewn with 4-0 Prolene. Inspection of all dista anastomoses and cannulation sites were noted to be hemostatically stable Inspection of the left internal mammary artery bed revealed it to be stable. Thi area was also copiously irrigated with bacitracin saline. The chest was flushe with warm normal saline and continuing antibiotic solution. Two #32 Frenc mediastinal chest tubes were placed. These were secured to the skin in th standard fashion. The chest was then closed utilizing interrupted #5 stainles steel wires, 0 Tycron, 0 Dexon, running 2-0 Dexon, and running 3-0 Dexon. Th lower extremity had been previous closed utilizing running 2-0 and 3-0 Dexon.

George J. Magovern, Jr., M.D.

By: Robert R. Lazzara, M.D.
DICT: 02-18-92
TRAN: 02-19-92
RPL/per